Anxiety Disorders (DSM-5-TR): Teaching Overview

Clinical Companion Knowledge Library: for counselors and counseling students

1. What they are

Anxiety disorders are conditions in which **fear (alarm about a present threat) and anxiety (dread about a possible future threat)** are excessive for the situation, last longer than is developmentally expected, and drive **avoidance or safety behaviors** that cause distress or impairment. Everyday worry and normal developmental fears are not disorders. The line is crossed when fear is out of proportion, persistent, and impairing, and is not better explained by a substance, a medical condition, or another mental disorder.

The DSM-5-TR anxiety chapter (teaching set): generalized anxiety disorder (GAD), panic disorder, agoraphobia, social anxiety disorder, specific phobia, separation anxiety disorder (children *and* adults), and selective mutism. The chapter also includes substance/medication-induced anxiety disorder, anxiety disorder due to another medical condition, and other specified/unspecified anxiety disorder.

Neighbors, not members: obsessive-compulsive disorder (OCD) and related disorders, and trauma- and stressor-related disorders (PTSD, acute stress disorder, adjustment disorders), now have **their own chapters**. They share fear and avoidance, and exposure-based CBT logic overlaps, but they are separate diagnoses.

Counselor framing (general clinical consensus): anxiety disorders are very common, often begin in childhood or early adulthood, tend to persist without treatment, and are highly treatable. Avoidance keeps fear going; approach is the core of treatment.

2. Core symptoms to watch for

The DSM-5-TR criteria are paraphrased here in plain language. Use the manual itself for formal diagnosis.

- **GAD:** hard-to-control, excessive worry about **many** life areas, present more days than not for **at least 6 months**, with **3 or more** of: restlessness/keyed up, easy fatigue, poor concentration or mind going blank, irritability, muscle tension, sleep disturbance (**only 1 needed in children**).
- **Panic disorder: recurrent, unexpected** panic attacks (an abrupt surge of intense fear that peaks within minutes with **4 or more** physical/cognitive symptoms such as pounding heart, breathlessness, dizziness, chest pain, numbness, unreality, or fear of dying or losing control), followed by **1 month or more** of persistent worry about more attacks or a maladaptive change in behavior (e.g., avoiding exercise). *Panic attacks* can also occur as a specifier with any disorder.
- **Agoraphobia:** marked fear or avoidance of **2 or more** of five situation types (public transportation, open spaces, enclosed places, lines or crowds, being outside the home alone) because escape or help might be hard to get. Usually lasts **6 months or more**. It is diagnosed independently of panic disorder.
- **Social anxiety disorder:** marked fear of situations where one may be **scrutinized**, with fear of acting in a way that will be negatively evaluated (embarrassment, rejection). Situations are avoided or endured with intense fear, usually for **6 months or more**. In children it must occur with peers, not only adults. *Performance only* is a specifier.
- **Specific phobia:** marked, immediate fear of a specific object or situation, usually **6 months or more**. Subtypes: animal, natural environment, blood-injection-injury (often with fainting), situational, and other.
- **Separation anxiety disorder:** developmentally excessive fear of separation from attachment figures (**3 or more** signs, such as distress at separation, worry about harm to loved ones, refusal to leave home or sleep alone, nightmares, physical complaints). Lasts **at least 4 weeks in youth** and typically **6 months or more in adults**.
- **Selective mutism:** consistent failure to speak in specific social settings (e.g., school) despite speaking elsewhere, lasting **at least 1 month** (not just the first month of school), and not due to a language or communication disorder.

Differential tips:

- Worry content: GAD is diffuse; social anxiety is about evaluation; OCD involves intrusive obsessions plus compulsions; illness anxiety is about disease; PTSD fear is linked to a trauma.
- Panic attacks are a symptom, not a diagnosis. Ask whether they are *unexpected* (panic disorder) or *cued* (phobia, social anxiety, PTSD).
- Distinguish shyness from social anxiety by impairment and avoidance; consider autism or language disorders in selective mutism.

Medical and substance rule-outs (general clinical consensus): anxiety can be produced or worsened by **hyperthyroidism**, cardiac arrhythmias, asthma/COPD, hypoglycemia, vestibular problems, and, rarely, pheochromocytoma. Ask about **caffeine** and energy drinks, nicotine, **stimulants** (prescribed ADHD medication, cocaine, methamphetamine), decongestants, corticosteroids, thyroid hormone, and cannabis. Also ask about **withdrawal** from alcohol, benzodiazepines, or other sedatives. New-onset anxiety in later life or prominent physical symptoms warrants a medical evaluation.

Suicide and comorbidity: anxiety disorders are linked to suicidal behavior. In a meta-analysis of 42 observational studies, people with anxiety disorders had higher odds of suicidal ideation (OR 2.89), attempts (OR 2.47), and suicide deaths (OR 3.34) (Kanwar et al., 2013). A longitudinal meta-analysis found anxiety to be a statistically significant but **weak** independent predictor of ideation and attempts, with the strongest links for PTSD (Bentley et al., 2016). **Teaching point (general clinical consensus):** screen for suicide routinely, especially with co-occurring depression or substance use; "just anxiety" does not mean low risk. Comorbid depression, alcohol or other substance use, and multiple anxiety disorders are common.

3. Frequently used assessment instruments

These are among the most widely used measures for screening, rating severity, and tracking progress in anxiety disorders. **Screening tools do not diagnose;** a positive screen calls for a clinical interview against DSM-5-TR criteria. Click an instrument's website to find it.

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
GAD-7 (Generalized Anxiety Disorder-7)	Screening and tracking generalized anxiety.	Self-report, 7 items, about 2 minutes	No (free)	None beyond general clinical training	PHQ Screeners site
APA DSM-5 anxiety severity measures	Separate brief scales for panic, social anxiety, specific phobia, agoraphobia, separation anxiety, and generalized anxiety (adult and age 11 to 17 versions).	Self-report, 10 items each	No (free from the APA for clinical use)	None beyond general clinical training	APA assessment measures
PSWQ (Penn State Worry Questionnaire)	Severity of chronic, hard-to-control worry (generalized anxiety).	Self-report, 16 items	No (free)	None beyond general clinical training	Original article
BAI (Beck Anxiety Inventory)	Anxiety severity, especially physical symptoms; ages 17 to 80.	Self-report, 21 items, 5 to 10 minutes	Yes	★ Level B	Pearson
STAI (State-Trait Anxiety Inventory)	Separates current (state) anxiety from long-standing (trait) anxiety.	Self-report, 40 items	Yes (license per use)	No special level listed by the publisher	Mind Garden
SCARED (Screen for Child Anxiety Related Disorders)	Children and teens; screens panic, generalized, separation, social, and school anxiety.	Child and parent forms, 41 items	No (free)	None beyond general clinical training	University of Pittsburgh (PDF)
MASC 2 (Multidimensional Anxiety Scale for Children, 2nd ed.)	Children and teens ages 8 to 19; self and parent forms.	Self- and parent-report, 50 items	Yes	★ Level B	MHS
PHQ-9 (Patient Health Questionnaire-9)	Screening for co-occurring depression, which is common with anxiety.	Self-report, 9 items, about 2 minutes	No (free)	None beyond general clinical training	PHQ Screeners site
C-SSRS (Columbia Suicide Severity Rating Scale)	Screening for suicide risk; "just anxiety" does not mean low risk.	Clinician- or staff-asked questions, a few minutes	No (free)	Free online training offered and recommended	Columbia Lighthouse Project

★ **= requires professional qualification to purchase or specialized training.** Publishers sell these tests only to qualified users. At Pearson, **Level B** generally means a master's degree in a related field with formal assessment training, or a health care license; **Level C** means a doctorate in psychology or a related field with formal assessment training, or state licensure in a related field (Pearson qualifications policy). MHS, PAR, and WPS use similar levels. Check each publisher's page before ordering. "Original article" links go to the published study that introduced a free scale that has no official website; the scale can usually be obtained there or from its authors.

4. What works: psychological treatments with the strongest evidence

Bottom line for class: CBT built around **exposure** is the first-line psychological treatment across the anxiety disorders, with the largest and most consistent evidence base. ACT and applied relaxation are credible alternatives for some disorders. Gains from CBT are durable for many clients, but long-term data are thinner than short-term data.

CBT: the reference standard

- **Against placebo (the strictest test).** In 41 placebo-controlled RCTs (N = 2,843), CBT had a moderate effect on the target disorder (g = 0.56) and roughly tripled the odds of response (OR 2.97). Effects were large for GAD and OCD and small-to-moderate for social anxiety and panic. Exposure-heavy protocols trended toward larger effects (Carpenter et al., 2018).
- **Durability.** Across 69 RCTs, CBT beat control conditions up to 12 months after treatment. Beyond 12 months, effects remained significant for GAD (g = 0.22) and social anxiety (g = 0.42) but not clearly for panic disorder, and most trials were of low quality (van Dis et al., 2020).
- **Digital delivery.** Across 64 trials, internet/computer CBT had a large effect versus control (g = 0.80) and matched face-to-face CBT in head-to-head studies (Andrews et al., 2018).

Core CBT techniques, disorder by disorder

- **Exposure (all disorders).** Repeated, planned approach to feared situations, sensations, or images while **dropping safety behaviors**. The *inhibitory learning* model emphasizes violating feared expectancies ("What did you predict? What actually happened?"), varying contexts, and tolerating fear rather than waiting for it to drop (Craske et al., 2014).
- **Specific phobia.** Exposure-based treatment produced large effects and outperformed placebo and other psychotherapies. **In vivo** exposure beat imaginal or virtual-reality exposure at post-treatment. More sessions predicted somewhat better outcomes, but single-session formats also work (Wolitzky-Taylor et al., 2008).
- **Panic disorder: interoceptive exposure.** Clients deliberately provoke feared body sensations (spinning, overbreathing, straw breathing) to learn they are not dangerous. A component network meta-analysis of 72 trials found **interoceptive exposure and face-to-face delivery** were associated with better efficacy and acceptability. **Muscle relaxation and virtual-reality exposure** were associated with *lower* efficacy. The best vs. worst "evidence-based CBT" packages differed greatly (OR for remission 7.69) (Pompoli et al., 2018). In a landmark multisite RCT (n = 312), CBT and imipramine each beat placebo (Barlow et al., 2000).
- **Social anxiety disorder: cognitive therapy and behavioral experiments.** In a network meta-analysis of 101 trials, **individual CBT** had the largest effect of any psychological treatment versus waitlist (SMD −1.19) and beat psychological placebo (Mayo-Wilson et al., 2014). Clark's cognitive therapy uses **behavioral experiments** (e.g., dropping safety behaviors and checking what others actually notice), attention shifting, and video feedback. It outperformed exposure plus applied relaxation (84% vs. 42% no longer met criteria) (Clark et al., 2006).
- **GAD: CBT, then third-wave and relaxation.** A network meta-analysis of 65 RCTs found CBT (SMD −0.74) and third-wave CBTs (−0.76) reduced GAD symptoms versus usual care. **Only CBT remained superior at 3–12 months**, so the authors called CBT the likely first-line therapy (Papola et al., 2024). An earlier meta-analysis found large effects of psychotherapy for GAD (g = 0.84), with signs that CBT outperformed applied relaxation in the longer term (Cuijpers et al., 2014).
- **Applied relaxation (AR).** AR teaches rapid, cue-controlled relaxation applied at the first signs of anxiety. It is a credible option for GAD: an acceptance-based behavior therapy and AR produced comparable, large improvements (Hayes-Skelton et al., 2013). However, relaxation's advantage over usual care disappeared when high-risk-of-bias trials were removed (Papola et al., 2024). In panic CBT, muscle relaxation was a weak component (Pompoli et al., 2018).
- **OCD (brief note):** exposure and response prevention (ERP), a form of CBT, has very large effects versus waitlist (1.31) and placebo (1.33) (Öst et al., 2015). Refer to OCD-specific training; this is not covered further here.

Other approaches

- **Acceptance and commitment therapy (ACT).** Across 39 RCTs, ACT beat control conditions (g = 0.57) and did not differ significantly from CBT (A-Tjak et al., 2015). In an RCT with mixed anxiety disorders (n = 128), ACT and CBT, both including exposure, improved similarly (Arch et al., 2012). ACT is a reasonable alternative.
- **Interpersonal psychotherapy (IPT).** IPT has weaker support for anxiety than for depression. For social anxiety disorder, cognitive therapy beat IPT on response at post-treatment (65.8% vs. 42.1%) and at 1 year (68.4% vs. 31.6%), although both beat waitlist (Stangier et al., 2011).

- **Children and adolescents.** CBT (often with parent involvement) increased remission from the primary anxiety diagnosis (49.4% vs. 17.8% waitlist; NNT about 3), but evidence of superiority over usual care was weak (James et al., 2020). In the CAMS trial (n = 488, ages 7–17, separation, generalized, or social anxiety), response rates were **80.7% for CBT plus sertraline**, 59.7% CBT, 54.9% sertraline, and 23.7% placebo (Walkup et al., 2008). **Selective mutism:** a small pilot RCT of integrated behavior therapy (graduated exposure to speaking, school-based, with parents and teachers) found 75% responders (Bergman et al., 2013).
- **Medication (high-level referral notes).** In a network meta-analysis of 89 GAD trials, duloxetine, pregabalin, venlafaxine, and escitalopram were efficacious with relatively good acceptability. Benzodiazepines and paroxetine worked but were poorly tolerated (Slee et al., 2019). SSRIs/SNRIs are common first-line choices, and benzodiazepines carry dependence and withdrawal risks (general clinical consensus). Refer for a medication evaluation when anxiety is severe or impairing, when there is a poor response to good-quality CBT, when comorbid depression is significant, or when the client prefers it.

5. Christian and spiritually integrated interventions: what the evidence shows

Models. Religiously or spiritually integrated CBT keeps the standard structure (psychoeducation, cognitive restructuring, exposure, homework) and draws on the client's own faith resources. The most common adaptations are using religious content in cognitive restructuring and motivation, and using religious practices (prayer, meditation, values, religious coping) to support behavioral and cognitive work (de Abreu Costa & Moreira-Almeida, 2022). For Christian clients, examples include Scripture-informed reappraisal of catastrophic thinking, framing exposure as courage and trust, and contemplative prayer as an acceptance practice.

Key findings:

- **Meta-analysis of faith-adapted CBT (depression and anxiety).** Sixteen RCTs showed statistically significant benefits, but trials were **small and at high risk of bias**, especially from researcher and therapist allegiance. The authors said larger rigorous trials are needed before firm recommendations (Anderson et al., 2015).
- **Broadest meta-analysis.** Across 97 studies, religion/spirituality-adapted therapy beat no treatment and modestly beat secular therapies (psychological g = 0.33). In the **most rigorous additive designs**, which test the same therapy with vs. without R/S content, outcomes were **equivalent on psychological distress** (g = 0.13, not significant) but **better on spiritual well-being** (g = 0.34) (Captari et al., 2018).
- **Religious/spiritual interventions across mental health outcomes.** In 23 RCTs, the clearest benefit was for **anxiety symptoms**, though protocols were highly diverse and not standardized (Gonçalves et al., 2015).
- **GAD trials (multifaith).** A spiritually based intervention delivered by a spiritual care counselor was comparable to CBT (response 63.6% vs. 72.3%; n = 22) (Koszycki et al., 2010). In a second pilot it outperformed supportive therapy and improved spiritual well-being (n = 23) (Koszycki et al., 2014).
- **Other faith-specific trials.** A 2-week internet spiritually integrated program for religious Jewish adults with *subclinical* stress and worry (n = 125) beat waitlist and was rated more credible than progressive muscle relaxation (Rosmarin et al., 2010). A 4-week **Christian meditation** program for Christian university students with repetitive negative thinking (n = 71) beat waitlist on perseverative thinking (Knabb et al., 2020). A primary-care RCT (n = 180, mostly Christian) found 5 minutes of in-person **Christian intercessory prayer** reduced GAD-7 scores more than a music control at 2 and 6 weeks (Jacobson et al., 2026). It was a single-session adjunct in a symptom-screened (not diagnosed) sample.

Honest summary for teaching:

1. Faith-integrated approaches appear **about as effective as standard treatment** for reducing anxiety in religious clients. They are **not consistently superior**, and their added value is mainly in **acceptability, engagement, and spiritual well-being**.
2. The evidence is **thinner for anxiety than for depression**. We found **no large, rigorous RCT of Christian-integrated CBT for a DSM-diagnosed anxiety disorder**. Most trials are small pilots, multifaith or other-faith protocols, subclinical or preventive samples, or short-term follow-up, with allegiance risk.
3. **Clinical consensus cautions:** keep exposure central. Prayer, Scripture reading, or seeking reassurance from clergy can function as **safety behaviors** or compulsions (watch for scrupulosity, which belongs in the OCD chapter). Avoid framing anxiety as spiritual failure (e.g., misuse of "do not be anxious" texts), which adds shame.
4. **Practice ethics (general consensus):** integrate faith only when the client prefers it and after a spiritual assessment. Stay within your competence and role, and never substitute integration for evidence-based structure or risk assessment.

6. Quick classroom takeaways

- Diagnose by **focus of fear**, **duration** (most ≥6 months; panic ≥1 month of concern; separation ≥4 weeks in youth; selective mutism ≥1 month), **avoidance**, and **impairment**. Always rule out medical and substance causes and screen for depression and suicide risk.
- **Exposure-based CBT** is first-line: in vivo exposure for phobias, interoceptive exposure for panic, cognitive therapy with behavioral experiments for social anxiety, and CBT for GAD. ACT and applied relaxation are alternatives.
- **Faith-integrated care** is a legitimate, client-centered option that performs about as well as standard care, with spiritual benefits. The evidence is still modest, so keep exposure at the core.
- Screen and track with the free GAD-7 and the APA anxiety severity measures; use the SCARED for children, and check for depression and suicide risk.

Sources note

All journal articles listed in References were checked against Crossref (Oct 28, 2026). Each DOI below is a live link to the publisher record. Assessment instrument links and purchase and qualification levels were checked on the publishers' and distributors' websites, or on Crossref for original articles (Nov 1, 2026).

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